Ellery Partners

Transaction services | Financial due diligence

Financial Due Diligence
Harveyville Healthcare

Summary of audited financial statements, quality of earnings and pro forma credit metrics, FY24 t

Prepared for Quillwood Credit Partners

Report date: 18 September 2026

Version 1.0 (final)

Strictly confidential

Reference: EP/FDD/HH/2026-014

Date: 18 September 2026

Synthetic - for demonstration. All names, entities and figures are fictional.
Confidential. Prepared for the addressee only and not to be relied on by any other party.

Borrower and target	Harveyville Healthcare Pty Ltd
ABN	92 207 130 560
Registered office	Level 8, 45 Phillip Street, Parramatta NSW 2150
Sponsor	Ardent Health Partners Fund II, managed by Ardent Health Partners Pty Ltd
Auditor	Lindqvist Marsh Chartered Accountants
Financial year end	30 June
Facility under consideration	A$72.0M senior secured term loan with an undrawn capex tranche, 48 months
Prepared by	Ellery Partners
Prepared for	Quillwood Credit Partners
Report date and version	18 September 2026, version 1.0 (final)

Report details

1. Purpose, scope and basis of preparation

Quillwood Credit Partners has asked Ellery Partners to summarise the audited financial statements of Harveyville Healthcare Pty Ltd (Harveyville or the group) for the years ended 30 June 2024, 2025 and 2026, to assess the quality of reported earnings and cash flow, and to set out pro forma credit metrics for the proposed acquisition facility. The facility is a senior secured term loan of A$72.0M with a 48 month tenor and an undrawn capex tranche. It will part fund the acquisition of 100% of Harveyville by Ardent Health Partners Fund II at an enterprise value of A$136.5M.

This report is a financial due diligence summary prepared for the lender. It is not an audit or a review under Australian Auditing Standards, and we express no opinion on the financial statements. We have relied on the audited statements, which carry unmodified opinions from Lindqvist Marsh Chartered Accountants for each of the three years, and on information supplied by management and the sponsor. We have not independently verified that information beyond the procedures described below.

The following documents were reviewed:

- Information memorandum dated 10 August 2026.
- Independent valuation by Carrick Whitlam Valuation Advisory Pty Ltd dated 28 August 2026, which assesses enterprise value at A$140.0M and the freehold property at A$38.5M.
- Quantity surveyor report by Pellegrino Doyle Quantity Surveyors Pty Ltd dated 4 September 2026, covering the theatre expansion programme.
- Audited financial statements for FY24, FY25 and FY26, and the accounting and normalisation schedules prepared by Brennan Tolliver Corporate Advisory Pty Ltd as financial adviser to the transaction.
- Legal advice summaries from Hartigan Voss Lawyers on leases and the conditional share sale agreement.

All figures are in Australian dollars. Amounts are in millions (A$M) unless stated, and minor differences are due to rounding. To align with the likely covenant definition, the summary is presented on a pre AASB 16 basis, so that cash rent on the four leased sites is treated as an operating cost within EBITDA and lease liabilities are excluded from net debt. We prepared this presentation from the audited statements and reconciled it to them. Management accounts for the period after 30 June 2026 are due on 30 September 2026, and the term sheet is scheduled for 12 October 2026. We will update this report if the management accounts change any conclusion.

2. Executive summary

Harveyville is a mature, profitable and cash generative operator of seven private day hospitals in New South Wales. FY26 revenue was A$122.0M and normalised EBITDA was A$19.5M, a margin of 16.0%. Revenue has grown at around 7% a year over the last two years and the EBITDA margin has been stable at between 15.9% and 16.3% across the three year period. Our principal findings are as follows.

- Earnings quality is sound. Normalisation adjustments total a net A$0.8M, or 4.1% of normalised EBITDA, and consist of identifiable one off items and a remuneration adjustment. We consider the adjustments reasonable.
- Cash conversion is strong. Operating cash flow before interest and tax was 100.5% of reported EBITDA in FY26 and 98.9% in FY25. Working capital has been stable at around 39 days of receivables.
- Pro forma net debt is A$70.2M, which is 3.6x normalised EBITDA. Cash interest cover is 2.9x on our indicative pricing assumptions. These metrics leave limited headroom, and a 20% fall in EBITDA would lift leverage to 4.5x and reduce interest cover to 2.3x.
- The loan to value ratio is 51.4% against the independent enterprise valuation of A$140.0M. The A$38.5M freehold property provides direct asset cover for about 53% of the facility limit.
- Sponsor equity of A$66.0M funds 47.8% of total uses, which is a meaningful contribution. The sources and uses allow A$1.5M for transaction costs, which appears tight against the 2.00% upfront fee alone (about A$1.44M). We recommend that the sponsor confirm how the balance of costs will be funded.
- The A$11.4M theatre expansion programme is not reflected in the historical or pro forma metrics. Its funding, timing and ramp up are the main forward looking uncertainty in the credit.

FY26 revenue	A$122.0M
FY26 normalised EBITDA	A$19.5M (16.0% margin)
Purchase enterprise value	A$136.5M (7.0x normalised EBITDA)
Independent valuation (EV)	A$140.0M (28 August 2026)
Facility limit	A$72.0M
Sponsor equity	A$66.0M
Pro forma net debt	A$70.2M
Net debt to EBITDA	3.6x
Loan to value ratio	51.4%
Interest cover	2.9x

Key financial and credit metrics

3. Business overview

Harveyville was founded in March 2012 by Dr Miriam Calloway and partners with the opening of the Parramatta day hospital. It acquired the Newcastle and Maitland hospitals in July 2017, opened the purpose built Wollongong hospital in November 2022, and completed the Gosford and Orange acquisitions in May 2024, bringing the network to seven sites. The group also operates in Penrith. A conditional share sale agreement with Ardent Health Partners was executed in May 2026.

The group runs 24 operating theatres and 9 procedure rooms and performs approximately 46,800 procedures a year across ophthalmology, gastroenterology, orthopaedics, ENT, plastic and reconstructive surgery, and pain management. Average theatre utilisation is 74%. Revenue is earned from facility fees paid by private health insurers and self-funded patients, with a smaller contribution from the Department of Veterans' Affairs. At FY26 revenue of A$122.0M, average facility revenue is about A$2,600 per procedure.

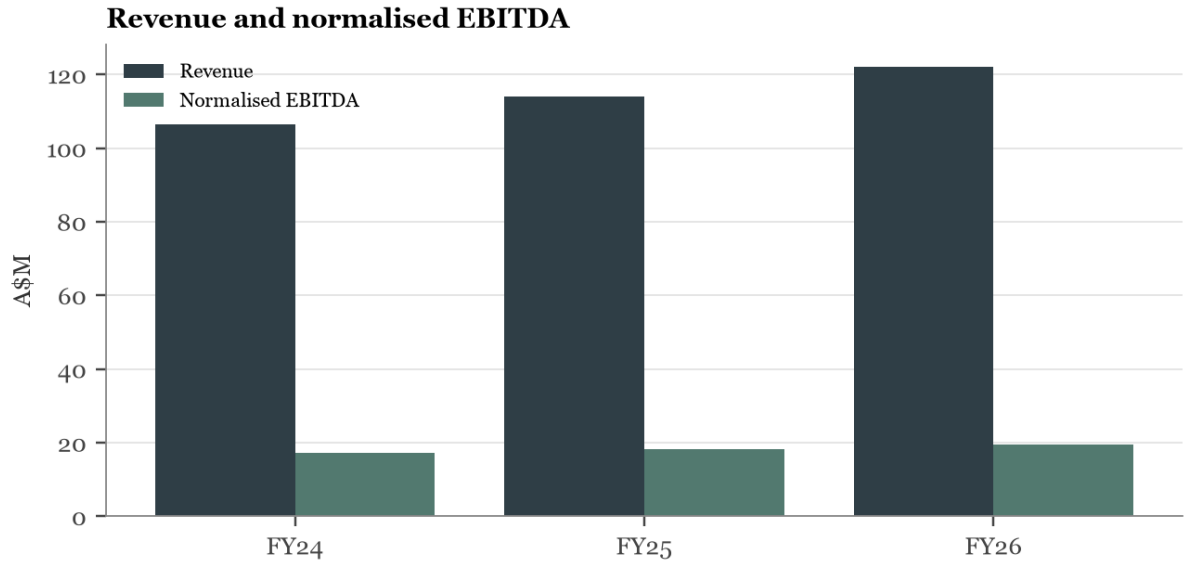
The group has 640 full time equivalent employees. Its 410 accredited doctors are independent visiting medical officers (VMOs) who are not employed by the group and pay no fixed fee. Three sites are freehold and four are leased on terms of 8 to 15 years including options. The board comprises Dr Miriam Calloway (Chair and Non-Executive Director), Stephen Okonkwo-Hale (Chief Executive Officer and Executive Director) and Priya Halloran (Non-Executive Director, Ardent Health Partners nominee).

4. Income statement and quality of earnings

4.1 Normalised operating performance

A$M	FY24	FY25	FY26
Revenue	106.4	114.1	122.0
Employee costs	(47.9)	(51.6)	(55.0)
Medical consumables and supplies	(21.3)	(22.8)	(24.4)
Occupancy (cash rent and outgoings)	(7.6)	(8.1)	(8.6)
Other operating costs	(12.3)	(13.4)	(14.5)
Normalised EBITDA	17.3	18.2	19.5
EBITDA margin (%)	16.3	15.9	16.0

Normalised summary income statement, years ended 30 June (pre AASB 16 basis)



Revenue and normalised EBITDA, FY24 to FY26

Revenue grew by 7.2% in FY25 and 6.9% in FY26. FY25 benefited from a full year of contribution from Gosford and Orange, which were acquired in May 2024 and so contributed for only around six weeks of FY24. FY26 growth is therefore a better indicator of underlying performance. The audited statements do not split growth between price and volume, and management has not yet provided a procedure volume bridge by site and specialty. We have requested it. Procedure volumes of 46,800 and a utilisation of 74% suggest some capacity remains, but we have not tested the sustainability of the growth rate.

Employee costs were 45.1% of revenue in FY26 (FY25: 45.2%; FY24: 45.0%), equal to an average of about A$86,000 per full time equivalent. Medical consumables and supplies have been steady at 20.0% of revenue. The stability of both ratios indicates that recent wage and supply cost increases have been recovered through facility fee indexation and procedure mix. The group has no fixed VMO payments, so its cost base is largely variable with respect to practitioner activity. It is, however, exposed to nursing and anaesthetic labour availability, which we discuss in section 9.

4.2 Reconciliation of reported to normalised EBITDA

FY26, A$M	Amount	Comment
Reported EBITDA	18.7	Pre AASB 16 basis
Vendor sale process costs	0.9	Advisers and data room, non recurring
Gosford and Orange integration	0.3	Systems migration and rebranding
Market rate remuneration	(0.4)	Founder and director pay set to market
Normalised EBITDA	**19.5**	**Used for all credit metrics**

Reconciliation of reported EBITDA to normalised EBITDA

We reviewed supporting invoices and engagement letters for the vendor sale costs and the integration costs, and agree that both are non recurring. The remuneration adjustment is a negative adjustment, which we regard as conservative. Net adjustments were A$0.8M in FY24, A$0.6M in FY25 and A$0.8M in FY26. We did not identify unsupported pro forma add backs, and we have not given credit for any run rate savings or for earnings from the expansion programme.

4.3 Reported profit

A$M	FY24	FY25	FY26
Normalised EBITDA	17.3	18.2	19.5
Net normalisation adjustments	(0.8)	(0.6)	(0.8)
Reported EBITDA	16.5	17.6	18.7
Depreciation and amortisation	(6.0)	(7.0)	(7.4)
EBIT	10.5	10.6	11.3
Net finance costs	(2.6)	(3.2)	(3.1)
Profit before tax	7.9	7.4	8.2
Income tax expense	(2.4)	(2.2)	(2.5)
Net profit after tax	**5.5**	**5.2**	**5.7**

Reported summary income statement, years ended 30 June (pre AASB 16 basis)

Depreciation rose from A$6.0M to A$7.4M over the period as the Wollongong hospital and acquired sites were fully reflected and capital expenditure continued. The effective tax rate has been close to 30%. Net finance costs relate to the existing bank facilities, which will be repaid on completion and replaced by the new facility. Historical interest cost is therefore not indicative of future debt service.

5. Balance sheet and net debt

A$M, at 30 June	2025	2026
Cash	3.4	4.1
Receivables	12.3	13.2
Inventories and other current assets	6.0	6.5
Property, plant and equipment	70.0	70.4
Goodwill and intangibles	41.0	41.0
Deferred tax assets	2.2	2.4
Total assets	134.9	137.6
Payables and accruals	(13.9)	(14.8)
Employee provisions	(8.5)	(9.0)
Borrowings	(41.5)	(37.5)
Deferred tax and make good provisions	(3.5)	(3.7)
Total liabilities	(67.4)	(65.0)
Net assets	**67.5**	**72.6**

Summary balance sheet (pre AASB 16 basis)

Property, plant and equipment at 30 June 2026 includes freehold land and buildings carried at A$31.8M and plant, equipment and leasehold improvements of A$38.6M. The independent valuation by Carrick Whitlam assesses the three freehold sites at A$38.5M, which is A$6.7M above book value. The uplift is unrecognised in the accounts and we have not relied on it for any balance sheet measure other than the loan to value analysis in section 7.

Goodwill and intangibles of A$41.0M relate to earlier acquisitions and were unchanged over the year. The group has no impairment indicators on our review, though the carrying amounts will be reset by acquisition accounting on completion. Net tangible assets were A$31.6M at 30 June 2026.

Working capital is stable. Receivables were equivalent to about 39 days of revenue at both balance dates, consistent with settlement terms from private health insurers. Inventories represent about 69 days of consumables expense at both dates. Existing borrowings were A$37.5M and net debt A$33.4M at 30 June 2026 (2025: A$41.5M and A$38.1M). The existing debt will be repaid from the acquisition proceeds. Employee provisions, mostly annual and long service leave, are A$9.0M and are not treated as debt.

6. Cash flow

A$M	FY25	FY26
Reported EBITDA	17.6	18.7
Working capital and other movements	(0.2)	0.1
Operating cash flow before interest and tax	17.4	18.8
Interest paid	(3.2)	(3.1)
Income tax paid	(2.1)	(2.6)
Net operating cash flow	12.1	13.1
Capital expenditure	(7.2)	(7.8)
Net repayment of borrowings	(3.8)	(4.0)
Dividends paid	(0.7)	(0.6)
Net change in cash	**0.4**	**0.7**

Summary cash flow statement (pre AASB 16 basis)

Operating cash flow before interest and tax was 98.9% of reported EBITDA in FY25 and 100.5% in FY26. This indicates earnings are backed by cash, with no material capitalisation of operating costs or build in receivables. Closing cash was A$3.4M in FY25 and A$4.1M in FY26.

FY26 capital expenditure was A$7.8M, made up of an estimated A$4.6M of maintenance expenditure (3.8% of revenue) and A$3.2M of growth expenditure on theatre and equipment upgrades. Maintenance capex is our estimate based on the fixed asset register and management's allocation, and we recommend the facility documents define it by reference to a floor and not rely on a management split. The business self funded its capital programme and repaid A$7.8M of debt over the two years while paying modest dividends.

7. Transaction funding and pro forma credit metrics

7.1 Sources and uses

Item	A$M	% of total
Senior secured term loan	72.0	52.2
Sponsor equity (Ardent Health Partners Fund II)	66.0	47.8
Total sources	138.0	100.0
Purchase of 100% of Harveyville (enterprise value)	136.5	98.9
Transaction costs	1.5	1.1
Total uses	**138.0**	**100.0**

Sources and uses at completion

The purchase price represents 7.0x FY26 normalised EBITDA. The independent valuation of A$140.0M represents 7.2x. The purchase price is A$3.5M below the independent valuation, which supports the sponsor's entry price. The proposed pricing is 3 month BBSW plus 5.50% per annum cash margin, a 2.00% upfront fee and a 1.00% per annum commitment fee on undrawn amounts, subject to credit approval. The upfront fee on A$72.0M is A$1.44M, which would absorb almost all of the A$1.5M transaction cost allowance. Legal, accounting, valuation and stamp duty costs would then be unfunded. We recommend that Quillwood Credit Partners require confirmation that any excess is funded by additional sponsor equity and not by drawing the facility.

7.2 Pro forma credit metrics

Metric	Value	Basis
Facility limit	A$72.0M	Drawn at completion
Pro forma cash	A$1.8M	Operating cash acquired
Net debt	A$70.2M	Facility less cash
Net debt to EBITDA	3.6x	A$70.2M / A$19.5M
Loan to value ratio	51.4%	A$72.0M / A$140.0M
Interest cover	2.9x	A$19.5M / A$6.8M
Freehold cover of facility	0.53x	A$38.5M / A$72.0M

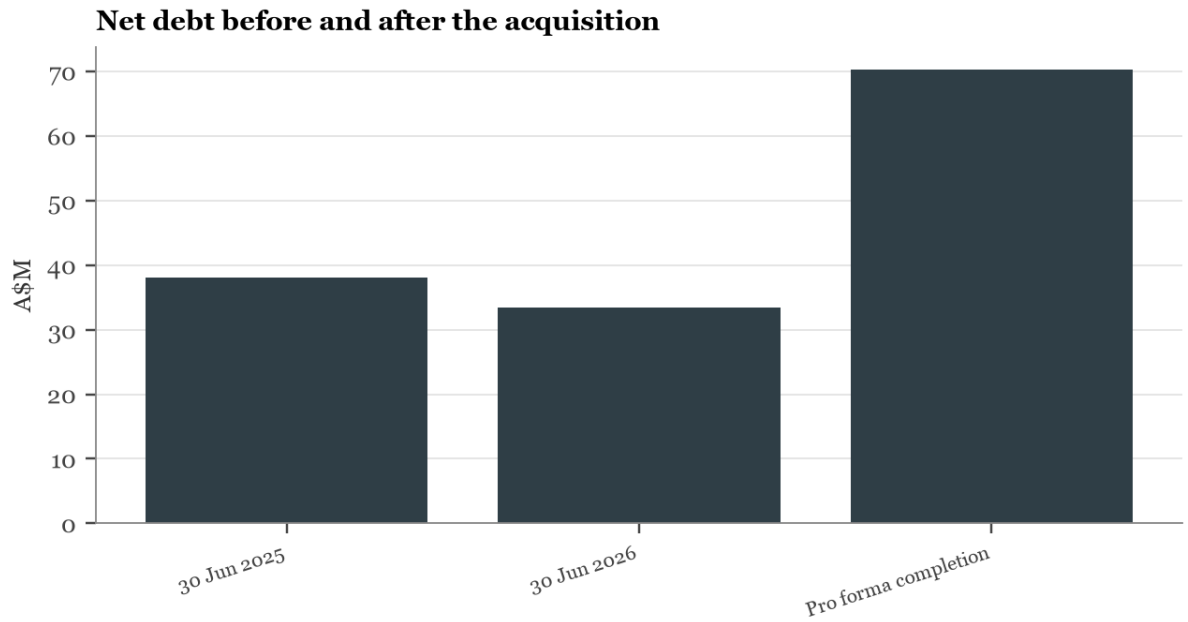
Pro forma credit metrics at completion

Cash interest of about A$6.8M assumes the facility is fully drawn at an all in rate of 9.4% per annum, being a 3 month BBSW assumption of 3.9% plus the 5.50% margin. The capex tranche is undrawn and its commitment fee is excluded. The tranche size and conditions are to be settled in the term sheet. Applying the same assumptions, cash flow available for debt service after maintenance capex of A$4.6M and notional cash tax of A$1.6M is about A$13.3M, or 2.0x cash interest. This leaves approximately A$6.5M a year for scheduled amortisation, growth capex and the expansion programme before any cash sweep.

7.3 Sensitivity

Scenario	EBITDA, A$M	Net debt / EBITDA	Interest cover
Base case	19.5	3.6x	2.9x
EBITDA down 10%	17.6	4.0x	2.6x
EBITDA down 20%	15.6	4.5x	2.3x
BBSW up 100 basis points	19.5	3.6x	2.6x
EBITDA down 10% and BBSW up 100 basis points	17.6	4.0x	2.3x

Sensitivity of leverage and interest cover (net debt held at A$70.2M)



Net debt rises from A$33.4M at 30 June 2026 to A$70.2M pro forma at completion

The sensitivities show that the structure absorbs a moderate EBITDA decline but with limited headroom. Net debt held constant ignores the cash the business would retain from lower tax and capex in a downturn, and also ignores any amortisation. We recommend covenant levels be set with at least 20% headroom to the sponsor's base case, to be informed by the sponsor financial model, which we have not yet received in final form.

8. Notes to the financial summary

Note 1. Basis of preparation

The audited statements are general purpose financial statements prepared under Australian Accounting Standards. For credit purposes we have adjusted them to a pre AASB 16 basis by deducting cash rent from EBITDA and excluding lease liabilities and right of use assets. The treatment of lease liabilities in net debt and covenants must be defined in the facility agreement.

Note 2. Revenue recognition

Facility fee revenue is recognised under AASB 15 when the procedure is completed and the patient is discharged. Revenue is stated net of insurer rebates and negotiated discounts. We found no material contract assets or deferred revenue. Revenue is not recognised for VMO professional fees, which are billed by the practitioners directly.

Note 3. Leases

Four of the seven sites are leased on terms of 8 to 15 years including options. Cash rent and outgoings are included in the occupancy cost line. Hartigan Voss Lawyers is reviewing the leases for change of control, assignment and rent review provisions, and we have not seen its final findings. Any landlord consent required on completion is a condition precedent matter for the lender.

Note 4. Freehold property

The freehold sites are carried at cost less depreciation. The Carrick Whitlam valuation of A$38.5M is a market value assessment and is not recognised in the accounts. A change in capitalisation rates would change this value, and a mortgage over each freehold site is expected to form part of the security.

Note 5. Employee provisions

Employee provisions of A$9.0M comprise annual leave and long service leave. We found no material underpayment or award compliance issues, though our work did not include a payroll audit.

Note 6. Contingent liabilities and commitments

The audited statements disclose no material contingent liabilities beyond claims in the ordinary course of operating healthcare facilities, which we understand are covered by insurance. Bank guarantees supporting leased premises are to be confirmed. The expansion programme of A$11.4M is not committed in the FY26 accounts.

Note 7. Related parties and events after balance date

Related party transactions disclosed are limited to director remuneration. We have asked management to confirm that no leased site has a related party landlord. We have identified no adjusting events since 30 June 2026. The conditional share sale agreement was executed in May 2026 and has not yet completed.

9. Key risks identified in diligence

- Regulatory and funding risk. Revenue depends on private health insurer rebates and negotiated hospital agreements, and is exposed to changes in Medicare Benefits Schedule items and insurer contract terms. We recommend the lender receive the principal insurer agreements, their expiry dates and the revenue split by payer before credit approval.
- Practitioner concentration. The top 25 VMOs generate approximately 38% of procedure volumes, or about 17,800 procedures a year. Loss of a few key surgeons could reduce theatre utilisation. We suggest information on the tenure and any restraint arrangements for these practitioners be requested.
- Leverage and debt service. At 3.6x leverage and 2.9x interest cover, headroom is limited if EBITDA falls or the base rate rises, as shown in section 7.3. Interest rate hedging for a substantial part of the term should be considered.
- Labour cost and availability. Shortages of nursing and anaesthetic staff may increase agency use and wages and compress the 16.0% EBITDA margin. Employee costs are 45.1% of revenue, so a one percentage point rise in that ratio would reduce EBITDA by about A$1.2M.
- Lease and property risk. Four of seven sites are leased, so renewals and rent reviews could change occupancy costs, and freehold values are sensitive to yield movements.
- Expansion execution risk. The A$11.4M theatre expansion programme could face cost overruns, delays or slower than forecast volume ramp up. The quantity surveyor's findings should be reconciled to the budget, and we recommend that drawings under the capex tranche be conditional on the QS certifying progress.

10. Matters outstanding and next steps

- Management accounts due 30 September 2026, to test FY27 year to date trading against the FY26 base.
- Procedure volume and price bridge by site and specialty, and revenue by payer.
- Final sponsor financial model including the expansion programme and tax effects of acquisition accounting.
- Confirmation of how transaction costs above the A$1.5M allowance will be funded.
- Final lease review from Hartigan Voss Lawyers, including landlord consents.
- Term sheet is scheduled for 12 October 2026, and covenant definitions should be settled by reference to the pre AASB 16 basis used in this report.

11. Limitations and sign-off

This report has been prepared solely for Quillwood Credit Partners in connection with its assessment of the proposed acquisition facility and must not be provided to or relied on by any other party without our written consent. It is based on information available at 18 September 2026. Forward looking figures, including pro forma interest, are illustrative and depend on assumptions that may not be realised.

Prepared by	Ellery Partners, Transaction Services
Engagement partner	Rhys Anselmi, Partner
Reviewer	Dana Whitcombe, Director
Date of issue	18 September 2026
Status	Final, version 1.0

Sign-off